

Advise the patient with schizophrenia and the patient's carer

- Educate carer/family and patient: the patient often lacks insight into the illness and may be hostile towards carers. S/he may have difficulty functioning, especially in high stress environments.
- Encourage carer to be supportive and avoid trying to convince patient that beliefs or experiences are false or not real. Avoid hostility and criticism towards the patient.
- Advise patient to avoid alcohol/drug use and encourage regular sleep routine. Emphasise importance of treatment adherence.
- Advise the patient to continue social/educational/occupational activities if possible. Refer to social worker to help find educational or employment opportunities.
- Consider housing/assisted living support and try to avoid long-term hospitalisation.
- Refer patient and carer to support group and cognitive behavioural therapy if available. Arrange support for carer and refer for therapy if available. Refer to community health worker.

Treat the patient with schizophrenia

- Give medication as in table below. Use lowest effective dose. Give one medication at a time. Allow 6 weeks on typical effective dose before considering medication ineffective.
- If repeated adherence problems, consider changing from oral to long-acting intramuscular medication (for health care workers with advanced psychiatric training). If possible, stabilise patient on oral antipsychotic agent before changing to IM depot preparation. Once stable on long-term depot, reduce oral formulation.
- If unsure or more than typical effective dose needed, discuss with specialist.

Medication	Starting dose	Maintenance dose	Note
Haloperidol	Start 1mg orally daily. If poor response, increase gradually to 5mg daily. If > 65 years start 0.75mg 12 hourly and increase more gradually.	Usually 5mg daily.	Minimal anticholinergic side effects ¹ . Monitor for extrapyramidal side effects (EPSE) ² : if present, switch to risperidone.
Risperidone	Start 2mg orally daily. If poor response after 4 weeks, increase to 4mg daily.	Usually 2-4mg daily.	• Use in patients with extrapyramidal side effects (EPSE)². • Use short term for breakthrough episodes. Discuss, if possible.
Flupenthixol decanoate	Start single dose 20mg IM. If poor response, give further 20mg IM after 1-2 weeks. If > 65 years: avoid use of IM antipsychotics, discuss with specialist.	Usually 20-80mg IM every 4 weeks.	• Full response can take 2 months. • Fewer anticholinergic side effects¹ than chlorpromazine.
Zuclopenthixol decanoate	Start single dose 100mg IM. If poor response, give further 200mg IM after 1-2 weeks. If > 65 years, avoid use of IM antipsychotics, discuss with specialist.	Usually 200-600mg IM every 4 weeks.	• Monitor for extrapyramidal side effects² (EPSE): if any EPSE develop, start orphenadrine 50mg 8 hourly and refer for specialist review.
Chlorpromazine	Start 25mg orally 12 hourly. If poor response increase at 25mg intervals.	Usually 75-300mg daily but 800mg may be needed. Once symptoms controlled, give as once daily bedtime dose.	• One of the most sedating antipsychotics. Avoid starting unless no other option. • Continue chlorpromazine only if patient stable on it and coping with any side effects.

Look for and manage schizophrenia treatment side effects

Urinary retention	Stop treatment, insert urinary catheter and refer same day.
Blurred vision	Stop treatment and refer same day.
Painful muscle spasms: acute dystonic reaction likely	Usually within 2 days of starting medication. Give biperiden 2.5mg IM. If needed, repeat after 30 minutes, up to 3 doses in 24 hours. Refer same day. If biperiden unavailable, give instead promethazine 50mg IM.
Abnormal involuntary movements	Stop treatment and discuss/refer same day. Doctor to consider switch to risperidone (above).
Muscle restlessness	Stop treatment and discuss/refer same day. Doctor to consider switch to risperidone (above).
Slow movements, tremor or rigidity	Discuss switch to risperidone (above) and arrange specialist review. Give orphenadrine 50mg 8 hourly whilst awaiting review.
Breast enlargement, nipple discharge, amenorrhoea	Discuss with specialist whether to change medication.
Dizziness/fainting on standing	Usually when starting/increasing dose. Usually self-limiting over hours to days. Advise to stand up slowly.
Dry mouth/eyes	Usually self-limiting.
Constipation	Usually self-limiting. Advise high fibre diet and adequate fluid intake.

Once stable, review 3 monthly. Advise to return immediately if symptoms of psychosis. If restarting treatment after interruption, review after 2 weeks, sooner if symptoms worsen.

¹Anticholinergic side effects include: urinary retention, blurred vision, dry mouth/eyes, constipation. ²Extrapyramidal side effects (EPSE) include: acute dystonic reaction (acute painful muscle spasm), abnormal involuntary movements, muscle restlessness, slow movements, tremor or rigidity.